SACTCHECK TREATMENT ASSESSMENT

Capecitabine

NCCP 00216 | 8
SACTCheck v0.83.4

TUMOUR GROUP	INDICATION	COURSE
Gastrointestinal	**Breast and colorectal cancer indications; 1250 mg/m^2 twice daily on days 1-14 every 21 days.**	**Course metadata not confirmed**

TREATMENT CONTEXT	ASSESSMENT ID	ASSESSMENT TIME
Not entered	**JSON-DEMO-001**	**16/10/2026, 23:34**

ENCODED CRITERIA RESULT

1 encoded criterion requires clinical review

Review the highlighted protocol criteria and the current NCCP source before the final treatment decision.

ENTERED VALUES AND ENCODED CRITERIA

Standard assessment

DOMAIN	ENTERED VALUE	ENCODED CRITERION	RESULT
Current diarrhoea grade	Grade 1	<= 1	**Dose-modification pathway**
Diarrhoea: occurrence of this toxicity (including this episode)	2	= 2	**Dose-modification pathway**
Highest diarrhoea grade in this episode	Grade 3	= 3	**Dose-modification pathway**

COMPONENT-SPECIFIC PROTOCOL ACTIONS

Protocol-level guidance; verify prescribed dose/current level

Capecitabine: Use 50% of documented starting dose - 50% of documented starting dose per administration; confirm prior reductions and pharmacy tablet banding
After recovery to grade 0-1, table 4 gives 50% of starting capecitabine dose. Reconcile earlier reductions and other findings before restart; do not re-escalate after a toxicity reduction. page 5 | Table 4

UNASSESSED DOMAINS

ECOG performance status, Hypersensitivity to capecitabine or its excipients, Previous severe, unexpected fluoropyrimidine reaction, Brivudine now or within the previous four weeks, Pregnant, Breastfeeding, DPD status, Assessment timing (optional) + 18 more

Blank domains were not assumed normal.

CLINICIAN DECISION

Not documented
Not documented

CLINICAL DECISION SUPPORT - NOT TREATMENT CLEARANCE

SACTCheck applies encoded NCCP protocol criteria to clinician-entered information. This output does not constitute treatment clearance and does not replace complete clinical assessment, the current official NCCP protocol, prescribing information, local policy or professional judgement. The responsible oncology clinician retains responsibility for the final treatment decision.